SLEEP-05 | Therapist only - all age groups

Daytime functioning and support

1 | Purpose, fit and boundaries

Purpose of this unit

Identify how tiredness or sleepiness affects daytime activities, and plan practical adjustments and support. Explore what is necessary and what can be reduced without presenting extra effort as the answer to every difficulty.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Do not drive or perform a dangerous task when sleepy. Unintentional sleep or ongoing difficulty functioning requires medical evaluation, not only task adjustments. Do not change medication, recommend stimulants, or prescribe avoiding rest.

Reusing existing material

SLEEP-02 addresses conditions and SLEEP-03 the transition to evening. This unit focuses on daytime functioning and support; EXEC can help break down a task once its scope is appropriate.

A fictional example for discussion

Fictional example: Tomer finds it hard to concentrate in class after an uncomfortable night. With an adult, he asks for written task instructions and lets staff know about the difficulty, instead of facing the demand to finish everything alone.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | A conversation about one situation

Opening

Choose a situation the person wants to explore. It need not be the hardest situation. Check that the question is understood and the wording fits the person; their account comes before the therapist's interpretation.

1 | Question for discussion

How does difficulty resting affect my daytime functioning?

2 | Question for discussion

Which tasks need particular alertness and safety?

3 | Question for discussion

What can be reduced, postponed, or reassigned by agreement?

4 | Question for discussion

What support will I need to arrange with others?

5 | Question for discussion

What change in functioning would I like to discuss in a medical evaluation?

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

If demands cannot be reduced, explore who has the authority to help. Do not treat an adjustment as giving up or demand activity as proof of ability despite sleepiness.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

The adult arranges support with the setting as needed. Do not attribute tiredness to unwillingness or ask the child to meet an unsafe demand.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

Allow requests for adjustments without unnecessary disclosure to peers. Explore road safety, activities, and responsibilities placed on the adolescent.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

Include driving, work requiring alertness, and caring for others. Practical arrangements do not replace medical evaluation, and this sheet does not certify fitness for an activity.

OA | Older adults

Preserve choice and participation according to preference without assuming dependence. Support may be temporary; a sudden change in functioning needs appropriate assessment.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

Choose an adjustment to a safe daily activity and agree on who will help. This trial does not test alertness, driving fitness, or the ability to manage without rest.

At the next session

Check whether support enabled safe functioning and choice. Leave unexplained difficulties for professional assessment, even if one adjustment helped.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

www.nhs.uk/conditions/insomnia/
www.nhs.uk/conditions/excessive-daytime-sleepiness-hypersomnia/

Mindful Path